EMERGENCY SURGERY · MIDTERM · COMPLETE QUESTION BANK

Emergency Surgery — Midterm All Lectures Question Bank

جراحة / طب الطوارئ — منهج الميدترم — مجمّع أسئلة المادة كلها

4	11	198	59
Lectures	Past exam questions	Expected Qs	Flashcards

#	Lectures included in this file · المحاضرات اللي في الملف	Past Qs	Expected	Cards
1	Acute Abdomen البطن الحادة	1	59	15
2	Acute Pancreatitis التهاب البنكرياس الحاد	4	39	14
3	Burn الحروق	2	50	15
4	Trauma الإصابات (Trauma)	4	50	15

المصادر:

كويز سابق (جراحة طوارئ)

ترقيم الأسئلة: رقم المحاضرة ثم رقم السؤال (مثلاً 4.12). الأسئلة المتوقعة عليها حرف N. الإجابات كلها في آخر الملف.

اللَّهُمَّ لا سَهْلَ إِلَّا ما جَعَلْتَهُ سَهْلًا، وأنتَ تَجْعَلُ الحَزْنَ إذا شِئْتَ سَهْلًا — صلِّ على النبي ﷺ

LECTURE 2

Acute Pancreatitis

التهاب البنكرياس الحاد

Part A · Past Exam Questions

أسئلة الدفعات السابقة

2.1 The most common cause of acute pancreatitis: كويز سابق (جراحة طوارئ)

a. Idiopathic
b. Alcoholism
c. Bile duct stone
d. Iatrogenic

2.2 The best treatment of acute pancreatitis is: كويز سابق (جراحة طوارئ)

a. Conservative treatment
b. CT guided aspiration
c. Laparotomy and debridement
d. None of the above

2.3 The most sensitive test for diagnosis of acute pancreatitis is: كويز سابق (جراحة طوارئ)

a. Serum calcium level
b. Urinary amylase level
c. Serum amylase level
d. Serum lipase level

2.4 As regards surgical drainage of pancreatic pseudocyst, all are correct EXCEPT: كويز سابق (جراحة طوارئ)

a. Indicated in all cases
b. Done by cystogastrostomy
c. Indicated in infection
d. Indicated if the cyst > 6 cm

Part B · Expected Questions (from the lecture)

أسئلة متوقعة على المحاضرة بنفس أسلوب الكويزات

2.N1 The pancreas is a/an: MCQ

a. Pelvic organ
b. Upper abdominal retroperitoneal organ
c. Mediastinal organ
d. Intraperitoneal organ

2.N2 The main pancreatic duct joins the common bile duct at the ampulla of Vater which opens into: MCQ

a. 3rd part of duodenum
b. 2nd part of duodenum
c. 1st part of duodenum
d. Jejunum

2.N3 The pancreas secretes daily about: MCQ

a. 100–200 mL
b. 1–2 litres
c. 5 litres
d. 500 mL

2.N4 The most common cause of acute pancreatitis is: MCQ

a. Trauma
b. Alcohol
c. Hypertriglyceridemia
d. Bile duct stones

2.N5 Excess alcohol intake causes about _____ of cases of acute pancreatitis: MCQ

a. 10%
b. 30%
c. 50%
d. 70%

2.N6 All of the following are causes of acute pancreatitis EXCEPT: MCQ

a. ERCP
b. Hypertriglyceridemia
c. Hypothyroidism
d. Hyperparathyroidism

2.N7 Acute pancreatitis can follow ERCP. TRUE / FALSE

a. True
b. False

2.N8 The pathogenesis of acute pancreatitis is: MCQ

a. Bacterial infection of the pancreas
b. Premature activation of pancreatic enzymes inside the gland leading to autodigestion
c. Pancreatic duct tumor
d. Insulin deficiency

2.N9 Inflammatory cytokines released in acute pancreatitis may lead to: MCQ

a. Hypertension
b. Polycythemia
c. Hyperkalemia
d. SIRS

2.N10 Hypocalcemia in severe acute pancreatitis is caused by: MCQ

a. Vomiting
b. Renal loss
c. Fatty acids binding calcium forming calcium soaps (fat necrosis)
d. Low vitamin D

2.N11 The pain of acute pancreatitis is: MCQ

a. Lower abdominal colicky pain
b. Epigastric pain radiating to the back, acute onset and gradually increasing
c. Pain referred to the left shoulder
d. Right iliac fossa pain

2.N12 Patients with acute pancreatitis often sit: MCQ

a. Prone
b. Supine
c. Trendelenburg
d. Leaning forward (prayer position)

2.N13 Local signs of acute pancreatitis are less impressive than expected from severity of pain because the pancreas is: MCQ

a. Retroperitoneal
b. Small
c. Intraperitoneal
d. Painless

2.N14 Cullen's sign is: MCQ

a. Bruising around the umbilicus
b. Pain in the right shoulder
c. Jaundice
d. Bruising in the loin

2.N15 Grey Turner's sign is: MCQ

a. Rigidity
b. Tenderness at McBurney's point
c. Bruising around the umbilicus
d. Bruising in the loin (flank)

2.N16 Upper abdominal swelling 2–3 weeks after an attack of acute pancreatitis suggests: MCQ

a. Pancreatic pseudocyst
b. Perforated ulcer
c. Gallbladder mucocele
d. Pancreatic cancer

2.N17 All of the following are systemic complications of acute pancreatitis EXCEPT: MCQ

a. ARDS
b. Shock
c. Paralytic ileus
d. Pancreatic abscess

2.N18 Renal failure in acute pancreatitis is due to: MCQ

a. Hypovolemia
b. Nephrotoxic enzymes
c. Stones
d. Hypercalcemia

2.N19 Tetany in acute pancreatitis is due to: MCQ

a. Hypokalemia
b. Hyponatremia
c. Hypocalcemia
d. Hyperglycemia

2.N20 Differential diagnosis of acute pancreatitis includes all EXCEPT: MCQ

a. Acute appendicitis
b. Acute cholecystitis
c. Myocardial infarction
d. Perforated peptic ulcer

2.N21 In acute pancreatitis serum amylase is usually elevated to more than: MCQ

a. 100 IU
b. 500 IU
c. 1000 IU
d. 10,000 IU

2.N22 Serum amylase is specific for acute pancreatitis. TRUE / FALSE

a. True
b. False

2.N23 The more specific enzyme for acute pancreatitis is: MCQ

a. LDH
b. Lipase
c. Amylase
d. ALT

2.N24 Arterial blood gases in acute pancreatitis are done to: MCQ

a. Measure amylase
b. Detect patients who will need mechanical ventilation
c. Detect stones
d. Diagnose pancreatitis

2.N25 **Laboratory findings in acute pancreatitis include all EXCEPT:** MCQ

a. Hypocalcemia
b. Elevated blood urea
c. Hypoglycemia
d. Hyperglycemia

2.N26 **Abdominal ultrasound in acute pancreatitis may show:** MCQ

a. Air under diaphragm
b. Gallbladder stones and dilated biliary tree
c. Kerley B lines
d. Pancreatic necrosis clearly

2.N27 **The investigation that shows pancreatic necrosis is:** MCQ

a. Plain X-ray
b. CT scan with IV contrast
c. Serum lipase
d. Ultrasound

2.N28 **MRCP is indicated in acute pancreatitis if:** MCQ

a. CBD stones are suspected
b. Always
c. Pseudocyst is present
d. Pancreatic necrosis is suspected

2.N29 **The first line treatment of acute pancreatitis is:** MCQ

a. Antibiotics only
b. ERCP in all cases
c. Conservative treatment (R regimen)
d. Urgent laparotomy

2.N30 **Pain in acute pancreatitis is relieved by pethidine combined with atropine to:** MCQ

a. Increase heart rate
b. Counteract spasm of the sphincter of Oddi
c. Reduce secretion only
d. Prevent vomiting

2.N31 **Rest of the pancreas in acute pancreatitis is achieved by:** MCQ

a. Keeping the patient NPO
b. Laxatives
c. Early ambulation
d. High-protein diet

2.N32 **Prophylactic antibiotics in acute pancreatitis are now used only in:** MCQ

a. Necrotizing pancreatitis (carbapenems preferred)
b. All patients
c. Mild pancreatitis
d. Alcoholic pancreatitis

2.N33 **An obstructing bile duct stone in acute pancreatitis is removed by:** MCQ

a. ESWL
b. Colonoscopy
c. ERCP
d. Laparotomy

2.N34 **Surgical treatment of acute pancreatitis includes all EXCEPT:** MCQ

a. Excision of necrotic tissue in severe necrotizing pancreatitis
b. Routine pancreatectomy for mild cases
c. Drainage of pancreatic abscess
d. Drainage of persistent pseudocyst

2.N35 **Pancreatic pseudocyst is lined by:** MCQ

a. Epithelium
b. Mucosa
c. Endothelium
d. Inflammatory tissue (false cyst)

2.N36 **Pancreatic pseudocyst develops after acute pancreatitis in about:** MCQ

a. 1% of cases
b. 10% of cases
c. 50% of cases
d. 90% of cases

2.N37 **Pancreatic pseudocyst is diagnosed by:** MCQ

a. CT scan
b. Barium meal
c. Plain X-ray
d. Serum amylase

2.N38 **Most pancreatic pseudocysts resolve spontaneously.** TRUE / FALSE

a. True
b. False

2.N39 **A pancreatic pseudocyst is surgically drained if it does not resolve within:** MCQ

a. 1 week
b. 2 weeks
c. 6 weeks
d. 6 months

Part C · Quizlet Flashcards

كروت مراجعة سريعة

غطّي العمود الأبيض بورقة واختبر نفسك: شوف المصطلح وقول التعريف.

Pancreas	Retroperitoneal; main duct joins CBD at ampulla of Vater (2nd part of duodenum); 1–2 L juice/day.	**Causes**	Gallstones 50%, alcohol 30%, idiopathic 20%; trauma/ERCP, hyperparathyroidism, hypertriglyceridemia.
Pathogenesis	Premature enzyme activation → autodigestion → cytokines → SIRS.	**Fat necrosis**	Lipase → fatty acids bind Ca^{2+} (calcium soaps) → hypocalcemia → tetany.
Pain	Upper abdominal, radiating to the back; patient leans forward; vomiting.	**Signs**	Fever, tachycardia, shock; Cullen's (periumbilical), Grey Turner's (flank) bruising.
Amylase	> 1000 IU, raised 2–3 days; not specific (perforated ulcer < 500).	**Lipase**	More specific (and sensitive) for acute pancreatitis.

Imaging	US: gallstones; CT with IV contrast: necrosis, edema, fluid; MRCP: CBD stones.
Local complications	Pancreatic pseudocyst, pancreatic abscess, necrosis.
Surgery	Excise infected necrosis, drain abscess, drain persistent pseudocyst.

Systemic complications	Shock, ARDS, renal failure, paralytic ileus, tetany.
R regimen	Relief of pain, Replacement of fluids, Respiratory support, Rest (NPO), Resistance to infection, Removal of stones (ERCP).
Pseudocyst	Juice + exudate without epithelial lining; 2–3 weeks after attack; most resolve; drain if > 6 weeks (cystogastrostomy).

1.N43. a — NSAIDs or corticosteroids

1.N44. b — 6:1

1.N45. c — Perforated peptic ulcer

1.N46. d — Erect chest X-ray showing air under the diaphragm
Gastrografin meal shows leakage of contrast.

1.N47. b — Graham's omental patch

1.N48. b — Lack of fibre leading to chronic constipation and increased intracolonic pressure

1.N49. c — Sigmoid colon

1.N50. b — False
CT with oral and IV contrast; barium enema is NOT done in the acute attack.

1.N51. a — Exploration with Hartmann's procedure
Pericolic abscess → U/S guided drainage.

1.N52. c — Strangulation intestinal obstruction

1.N53. b — Atrial fibrillation or mural thrombus after MI

1.N54. d — Maximum symptoms with minimal signs
Until peritonitis occurs.

1.N55. d — CT angiography

1.N56. b — 24 hours
Viable bowel: embolectomy (embolism), bypass graft (thrombosis), anticoagulation (venous).

1.N57. b — Anticoagulation therapy

1.N58. c — Stones of the urinary tract
Slide 33.

1.N59. d — Air under the diaphragm

2.1. c — Bile duct stone
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

2.2. a — Conservative treatment
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

2.3. d — Serum lipase level
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

2.4. a — Indicated in all cases
إجابة مقترحة (المصدر مفيهوش إجابة رسمية). معظم الـ pseudocysts بتختفي لوحدها، والتصريف بيكون للكيس اللي فضل أكتر من 6 أسابيع أو كبير أو حصله infection.

2.N1. b — Upper abdominal retroperitoneal organ
Slide 2.

2.N2. b — 2nd part of duodenum

2.N3. b — 1–2 litres
Alkaline enzyme-rich juice (amylase, lipase).

2.N4. d — Bile duct stones
Bile duct stones 50%, alcohol 30%, idiopathic 20%.

2.N5. b — 30%

2.N6. c — Hypothyroidism

2.N7. a — True
Trauma: accidental, operative or during ERCP.

2.N8. b — Premature activation of pancreatic enzymes inside the gland leading to autodigestion

2.N9. d — SIRS

2.N10. c — Fatty acids binding calcium forming calcium soaps (fat necrosis)

2.N11. b — Epigastric pain radiating to the back, acute onset and gradually increasing

2.N12. d — Leaning forward (prayer position)

2.N13. a — Retroperitoneal

2.N14. a — Bruising around the umbilicus

2.N15. d — Bruising in the loin (flank)

2.N16. a — Pancreatic pseudocyst

2.N17. d — Pancreatic abscess
Pancreatic abscess and pseudocyst are LOCAL complications.

2.N18. a — Hypovolemia

2.N19. c — Hypocalcemia

2.N20. a — Acute appendicitis
DD: perforated PU, acute cholecystitis, MVO, leaking aortic aneurysm, MI.

2.N21. c — 1000 IU
Remains elevated for 2–3 days.

2.N22. b — False
Not specific — may rise in perforated peptic ulcer (usually < 500 IU).

2.N23. b — Lipase

2.N24. b — Detect patients who will need mechanical ventilation

2.N25. c — Hypoglycemia

2.N26. b — Gallbladder stones and dilated biliary tree

2.N27. b — CT scan with IV contrast

2.N28. a — CBD stones are suspected

2.N29. c — Conservative treatment (R regimen)

2.N30. b — Counteract spasm of the sphincter of Oddi

2.N31. a — Keeping the patient NPO
New concept: IV fluids until nausea and vomiting stop.

2.N32. a — Necrotizing pancreatitis (carbapenems preferred)

2.N33. c — ERCP

2.N34. b — Routine pancreatectomy for mild cases

2.N35. d — Inflammatory tissue (false cyst)

2.N36. b — 10% of cases
2–3 weeks after the attack.

2.N37. a — CT scan
Often painless and discovered on follow-up US.

2.N38. a — True

2.N39. c — 6 weeks
Internally into the stomach (cystogastrostomy) or jejunal loops.

3.1. d — 18% burns
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

3.2. a — Chemical burn
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

3.N1. b — Second (after muscles)

3.N2. b — Stratified squamous epithelium
Dermis = connective tissue.

3.N3. d — Lymph nodes

3.N4. a — True

3.N5. a — Formation of vitamin K

3.N6. c — Boiled liquids
Often affect children.

3.N7. d — Ignition of clothing

3.N8. c — Blood and bones are good conductors

3.N9. b — Hot gases
May affect upper or lower airway.

3.N10. d — Rule of 9
Modified in children (large head).

3.N11. c — 30%

3.N12. b — 15–30%
10–30% in children.

3.N13. b — 10%
< 15% in adults.

3.N14. b — 9%

3.N15. a — 1%

3.N16. c — Sunburn
Erythema only; heals rapidly.

3.N17. c — 3 weeks

3.N18. a — True

3.N19. a — Complete destruction of epidermis and dermis

3.N20. c — Hypertension as the main problem

3.N21. c — Pain

3.N22. b — Prolonged uncorrected hypovolemia

3.N23. a — Acute ulceration of the stomach and duodenum after burns
Rare now with antacids and H2 blockers.

3.N24. b — Days 4–7 postburn

3.N25. a — True
Prevention is by proper local wound care.

3.N26. a — Urgent escharotomy

3.N27. a — Urgent tracheostomy

3.N28. d — Malignant transformation in a long-standing burn scar

3.N29. b — Curling's ulcer
Curling's ulcer is SYSTEMIC.

3.N30. c — 15 minutes
Saline or tap water at room temperature.

3.N31. b — May induce more tissue damage

3.N32. a — True
< 15% adults, < 10% children: dressing + analgesia.

3.N33. d — First-degree sunburn of 5%
Also > 15% adults / > 10% children and burns with trauma or comorbidity.

3.N34. d — Check urine output

3.N35. b — 4 mL/kg/% burn lactated Ringer's per day

3.N36. c — The first 8 hours
1/4 in the second 8 hours, 1/4 in the third 8 hours.

3.N37. b — 50%
Otherwise serious over-infusion may occur.

3.N38. c — 48 hours

3.N39. b — 0.5–1 mL/kg/hour

3.N40. b — False
Enteral nutrition is preferred.

3.N41. a — Povidone iodine

3.N42. c — Toxic to viable cells

3.N43. d — Face, neck and perineum
And burns of one side of the trunk.